

# ALZHEIMER'S DETECTION MEDICAL ANALYSIS REPORT

Report Generated: January 15, 2026 at 01:27  
Report ID: 20260115012740  
Analysis Method: Deep Learning (EfficientNet-B0)

## PATIENT INFORMATION

|               |            |
|---------------|------------|
| Patient Name: | John Doe   |
| Patient ID:   | P-2024-001 |
| Age:          | 65         |
| Gender:       | Male       |
| Scan Date:    | 2026-01-15 |

## ANALYSIS RESULTS

### MILD ALZHEIMER'S

| Classification        | Probability | Confidence |
|-----------------------|-------------|------------|
| Mild Alzheimer's      | 65.00%      | High       |
| Moderate Alzheimer's  | 20.00%      | Low        |
| Non-demented          | 10.00%      | Very Low   |
| Very Mild Alzheimer's | 5.00%       | Very Low   |

## MRI SCAN IMAGE



### CLINICAL INTERPRETATION

The scan shows patterns consistent with **mild Alzheimer's disease** with high confidence (65.0%). Noticeable structural changes are present, including potential hippocampal atrophy and ventricular enlargement. Clinical correlation with cognitive assessment is essential. At this stage, pharmacological interventions and lifestyle modifications may help slow disease progression.

### MEDICAL RECOMMENDATIONS

#### Recommendations:

- Immediate consultation with neurologist required
- Comprehensive cognitive and functional assessment
- Discuss pharmacological interventions (cholinesterase inhibitors)
- Implement structured daily routines
- Consider caregiver support and education
- Regular monitoring every 2-3 months
- Evaluate safety at home and driving capability

## IMPORTANT DISCLAIMER

**This report is generated by an AI-based analysis system and is intended for informational purposes only.** It should NOT be used as a sole basis for clinical diagnosis or treatment decisions. The results must be interpreted by a qualified healthcare professional in conjunction with clinical examination, patient history, and other diagnostic tests.

The AI model has been trained on MRI scan data but may not account for all clinical variables. False positives and false negatives are possible. Always consult with a board-certified neurologist or radiologist for definitive diagnosis and treatment planning.

## REVIEWED BY

**Doctor:** Dr. Jane Smith

**Role:** Neurologist

**Date:** January 15, 2026 at 01:27